Armadale Kalamunda Group

Bowel Management Guideline

1. Purpose

To establish minimum practice standards for bowel management across the Armadale Kalamunda Group.

2. Applicability

This guideline applies to nursing, midwifery and medical staff across Armadale Kalamunda Group.

3. General information

- Relevant bowel management standards should be applied in accordance with the needs of the individual patient.
- Continence issues must be identified on admission and through the ongoing monitoring of patients by medical, nursing, midwifery and allied health staff.
- Conducting a bowel assessment in the first instance will identify specific symptoms and considerations which may require specialised assessment but will also provide information that can be used to formulate a bowel management plan.
- The clinical status of the patient must be reviewed following all interventions and disease processes and in accordance with medical instructions.

4. Guidelines

4.1 Considerations

- All patients with bowel dysfunction must have a bladder assessment completed.
- Liaise with the MO and inform the shift coordinator if any of the following red flag symptoms are identified:
    - Cramping.
    - Confusion.
    - Delirium.
    - Fever.
    - Pain, including abdominal or lower back pain that is new or worsening.
    - Rectal bleeding or pain.
    - Presence of mucous / blood in stool.
    - Urinary incontinence that is new or worsening.
    - Urinary tract symptoms, such as pain or decreased flow.

Caution: The above symptoms may be indicative of intestinal failure whereby the intestines cannot digest food and absorb the fluids, electrolytes and nutrients essential to live. If any of the symptoms are indicating intestinal failure, refer to Appendix 1 Management of Intestinal Failure Guidelines.

4.2 Do not perform digital rectal examination or administer PR medication to:

- Neutropaenic or thrombocytopaenic patients.
- Patients who have had recent colorectal surgery or trauma.
- Patients who have had post radical prostatectomy.
- Patients who have had a low resection may be at risk of perforation.
- The medical officer may:
  - Perform a digital rectal examination.
  - Organise an abdominal X-ray.
  - Initiate further investigation e.g. colonoscopy, sigmoidoscopy, barium enema, faecal specimen or blood tests.

4.3 Baseline assessment

- Determines current bowel function and assists in identifying if further assessment is required.
- It is essential to establish whether or not the patient has a bowel dysfunction on admission or it is identified during their stay within the health care facilities.
  - Should any concerns about bladder function be identified refer to the Armadale Kalamunda Group (AKG) Bladder Management – Continence Procedure

4.4 Health history

History taken needs to include the patients' usual bowel activity including:

- Frequency of bowel action
- Stool consistency (refer to the Bristol Stool Chart)
- Straining / presence of urge.
- Presence of faecal incontinence.
- Triggers / dietary management.
- Level of activity.
- Any medications that may contribute to changes in bowel function.
- Use and frequency of use of laxatives or aperients.

4.5 Physical assessment

- Physical assessment by a medical officer / skilled nurse or midwife should include inspection, auscultation, percussion and palpation of the abdomen.
- Observation and assessment of the perianal and perineal areas for anal fissure, haemorrhoids, rectal prolapse and skin condition.
- Hard faecal masses in the colon may be felt.
- Pain related to haemorrhoids or anal fissures may contribute to constipation.

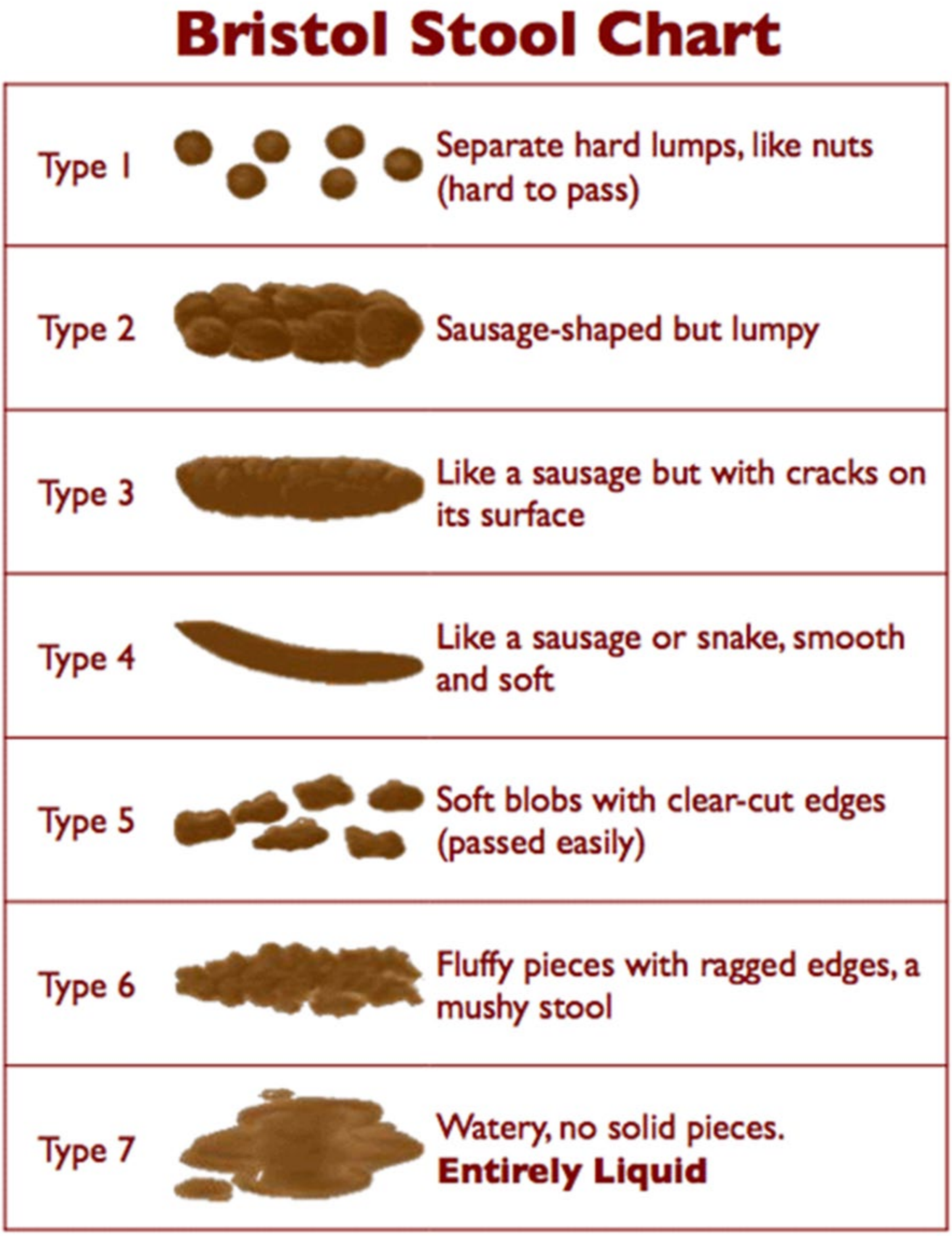
4.6 Documentation

- Failure to accurately and legibly record, and understand what is recorded, in the patient health records contribute to a decrease in the quality and safety of patient care.
- Assessment and bowel management plan to be documented in the patient health care record and site specific endorsed records.
- Monitoring of food and fluid intake will provide relevant information on intake and enable monitoring of the current bowel pattern, and assist in decision making processes for management strategies.
- Early identification and intervention of constipation will assist in reducing the risk of faecal impaction and urinary retention during periods of illness.
    - Commence a diet chart and fluid balance chart as required.
- Patients who require further investigation / management to be referred to the specialist continence service or liaise with the MO for an individualised management plan to be developed and implemented for inpatients and follow up as outpatient as required.

Refer to a continence service and / or the MO for the following:

- Patients with newly identified faecal incontinence.
- Patients with existing faecal incontinence who request further support and advice.
- Perineal dermatitis relating to incontinence.
- Chronic constipation / faecal impaction.

Bristol Stool Chart



5. Constipation

Constipation is determined following a bowel assessment and can be characterised by straining on defecation, reduced or infrequent defecation (<three per week) or a feeling of incomplete emptying of the rectum.

Constipation can be defined in three ways:

- **Normal transit:** stool moves through the colon normally but results in straining and hard stools.
- **Slow transit:** colonic inertia causes the stool to move through the colon slowly and results in hard stools and infrequent bowel actions.
- **Pelvic floor dysfunction:** resulting incomplete evacuation and obstruction.

Faecal impaction: is a state in which the person becomes so severely constipated that they are unlikely to be able to pass faeces of their own accord.

- It is usually but not necessarily, associated with hardened stools and patient discomfort.
- It is a major cause of faecal incontinence.

5.1 Assessment / management

- Establish when usual bowel activity last occurred and document on an AKMR141 Bowel Chart.
- Refer to fluid balance chart to evaluate if fluid intake adequate.
  - Promote 2L per day as clinically appropriate.
- Promote well balanced diet and monitor dietary intake as appropriate.
  - Liaise with the dietitian as indicated.
- Promote physical activity / exercise as able.
  - Consider referral to the Physiotherapy especially if immobile.
- The management of constipation depends on the cause, with many patients being affected by more than one causative factor.
  - Consider referring to the continence service.
- Faecal impaction should be confirmed following assessment by the MO.
  - Faecal impaction can be associated with faecal incontinence (also known as overflow).
  - Refer to the section Baseline Assessment.
- Oral preparations should be considered first and may be used in conjunction with rectal preparations:
  - Refer to Pharmacological Agents:
    - Laxatives / Aperients.
    - Rectal Preparations: Suppository.
    - Rectal Preparations: Enema.

5.2 Manual evacuation

- If medical therapies are unsuccessful, manual evacuation may be appropriate and should only be performed following liaison with the MO, by a person experienced in this procedure, in the presence of a chaperone and with informed patient consent.
- If manual evacuation is recommended by the MO, contact the continence services as required.
  - For information relating to manual evacuation in the patient with a spinal injury refer to site specific guidelines.

5.2.1 Manual evacuation should not be performed if the patient has:

- Had recent anal / rectal surgery.
- Rectal / anal trauma.
- Neutropaenia.
- Thrombocytopaenia.

Extra caution should be observed with patients who have:

- Rectal / anal pain.
- Obvious rectal bleeding.
- Anticoagulant medication.
- Active inflammatory bowel disease.
- Spinal injury at or above T6.
- Recent radiation treatment to the pelvic area.
- History of sexual abuse.

5.3 Documentation

- Assessment, treatment and outcomes of the patient's bowel management are to be monitored and documented.
- Failure to accurately and legibly record, and understand what is recorded, in patient health records contribute to a decrease in the quality and safety of patient care.

6. Diarrhoea

- Diarrhoea can be characterised by decreased stool consistency, increased frequency, urgency or volume may also be indicative of diarrhoea.
- Diarrhoea is a symptom and the underlying cause should be identified and treated if possible.

6.1 Acute diarrhoea: is a rapid onset and present for less than two weeks.

- Episodes of acute diarrhoea should be considered potentially infectious until otherwise proven.

6.2 Chronic diarrhoea: is persistent and has been present for more than two weeks.

- Symptoms can range from abdominal cramping to severe abdominal pain and there may be burning and / or itching around the perianal area. Refer to Skin Care: Incontinence Associated Dermatitis (IAD).

6.3 Risk Factors

Severe or extended diarrhoea may result in dehydration, electrolyte imbalance and malnutrition, anxiety, stress, and depression.

Risk factors for diarrhoea include:

- Recent overseas travel.
- Surgery e.g. abdominal and colorectal.
- Infection.
- Gastrointestinal tract disorders.

- Mal absorption syndromes.
- Lifestyle e.g. excessive alcohol intake.
- Psychological factors / psychiatric history.
- Medications including over the counter and/or complementary therapies.
- Allergies
- Graft versus host disease (GVHD).
- Medications which may cause diarrhoea include but are not limited to:
    - Antibiotics.
    - Non-steroidal anti-inflammatories.
    - Laxatives.
    - Antihypertensives.
    - Antiarrythmics.
    - Bronchodilators.
    - Antineoplastics.
    - Chemotherapy.
    - Metformin.

6.4 Assessment / management

- Assess when normal bowel activity last occurred and document an AKMR141 Bowel Chart.
- Refer to the Bristol Stool Chart to determine stool assessment including:
    - Frequency.
    - Consistency.
    - Colour.
    - Volume, mucous, blood, pus, excessive fats, undigested food or tablets and offensive odour.
- Monitor and document intake and output to identify problems such as dehydration on a fluid chart.
- Consider isolation of patients who have two or more unexplained loose bowel actions and send off stool specimen.
- Assess and record episode of:
    - Pain or discomfort.
    - Vomiting.
    - Generalised weakness.
    - Assess perianal area for signs of impaired skin integrity.
- Treatable factors of diarrhoea or faecal incontinence should be addressed prior to initiation of a bowel management plan.
- Initiate review by the MO who may consider:
    - Analgesia.
    - Anti-motility medications.
    - Antiemetics.
    - Intravenous (IV) therapy / electrolyte replacement.

    - Stool specimen collection for microbiology, culture and sensitivity (if not already undertaken).
    - Digital rectal examination.
    - Abdominal X-ray.
- Liaise with the continence service for further advice.
    - Consider the use of products such as faecal collection bags, skin care products.
- Consider a dietitian review in liaison with the MO for:
    - Review of nutritional status e.g. dietary intake.
    - Review of causative factors e.g. food sensitivity, enteral feeding.

6.5 Documentation

- Assessment, treatment and outcomes of the patient’s bowel management are to be monitored and documented.
- Failure to accurately and legibly record, and understand what is recorded, in patient health records contribute to a decrease in the quality and safety of patient care.

7. Faecal incontinence

Faecal incontinence can be defined as the involuntary passage of solid or liquid faeces and / or flatus.

7.1 Causes

- Faecal impaction with overflow.
- Anal sphincter / pelvic floor damage.
- Degenerative neurological disease i.e.: Alzheimer’s dementia.
- Ano-rectal pathology such as rectal prolapse.
- Gut motility / stool consistency i.e. infection, pelvic irradiation, irritable bowel syndrome, inflammatory bowel disease, dietary intake.
- Environmental / lifestyle, e.g. poor toileting facilities, physical / mental impairment.
- Idiopathic

Damage to the anal sphincter or pelvic floor may be caused by:

- Obstetric trauma.
- Anal trauma or surgery.
- Neurological disease (multiple sclerosis, spinal cord injury, cerebrovascular accident).

7.2 Risk factors

- Age.
- Gender.
- Constipation / faecal impaction.
- Diarrhoea.
- Cognitive impairment.

7.3 Assessment / management

- Assess when normal bowel activity last occurred and document on an AKMR141 Bowel Chart.
- Refer to the Bristol Stool Chart to determine stool assessment including:
    - Frequency.
    - Consistency.
    - Colour.
- Special management is required for those individuals:
    - Who are severely or terminally ill.
    - With spinal cord injury.
    - With intellectual disability.
- Liaise with the MO to treat any potentially reversible causes such as:
    - Faecal loading.
    - Treatable causes of diarrhoea e.g.: infective, inflammatory bowel disease.
    - Rectal prolapse.
    - Acute anal injury.
    - Acute injury such as disc prolapse
- Consider alternatives to medications that may be contributing to faecal incontinence.
- Refer to the continence service as required.
- Refer to Bowel Management Products section

Promote regular time for defecation:

- As per the patient's usual time or 30 minutes after meals.
- Encourage the patient to respond immediately to defecate.
- Encourage the patient to adopt the correct sitting position on the toilet and to avoid straining.
- Ensure there is appropriate dietary intake to promote ideal stool consistency.
- Consult with a dietitian as appropriate.

7.4 Documentation

- Assessment, treatment and outcomes of patient's bowel management are to be monitored and documented.
- Failure to accurately and legibly record, and understand what is recorded, in patient health records contribute to a decrease in the quality and safety of patient care.

8. Pharmacological agents

- The information on pharmacological agents provided in this guideline is not comprehensive and that the product information (including precautions and contraindications) should be referred to before initiating treatment.

- Laxatives are associated with increased bowel movements, frequency and improvement in symptoms of constipation.

- Use of pharmacological agents should depend on the nature, severity, cause, onset of action required and patient preference. Advice should be sought from a Pharmacist if required.
- The use of laxatives should only be considered when other options such as diet and fluid intake have been assessed and refined appropriately.
    - Laxatives / aperients should be introduced in a stepwise approach as follows.
    - Evaluate effectiveness of therapy and progress to next option if bowels not opened.

8.1 Bulking agents – seek advice from Pharmacist as may not be appropriate for all hospitalised patients

(Ispaghula husk i.e. Fybogel®; Psyllium husk; Guar gum i.e. Benefiber®)

- Absorption of water causes expansion which both increases the bulk of the stool and softens stool consistency. The increased bulk promotes peristalsis.
- Useful for both diarrhoea and constipation.
- Should be taken with a glass of water as inadequate fluid intake whilst using bulking agent can contribute to constipation, especially in the elderly population.
- Onset of action is one - three days.

8.2 Stimulant laxatives

(e.g. Bisacodyl – Durolax® tablets; Coloxyl and Senna® tablets - may be nurse/midwife initiated).

- Not recommended for long term use as stimulant laxatives provoke an irritant effect to stimulate intestinal motility.
- Stimulation of the nerve endings in the muscular wall of the colon causes increased peristalsis and secretion of fluid into the colonic lumen.
- Long-term use may cause bowel dependence.
- Onset of action is 15-60 minutes.

8.3 Osmotic laxatives

(Movicol; Sorbitol e.g. Sorbilax®; Lactulose e.g. Actilax®, Duphalac®; Magnesium salts e.g. Epsom Salts®)

- Magnesium salts are poorly absorbed and hold fluid in the faecal mass.
- Increased absorption of magnesium may cause toxic effects.
- Onset of action 24-72 hours.

8.4 Iso-osmotic laxatives

(e.g. Macrogol 3350, Movicol®)

- Increase the stool volume which promotes colonic activity and defecation.
- Maintains fluid in the faecal mass by osmosis.
- Contains electrolytes which ensure that there is no net loss of sodium, potassium or water.
- Onset of action 30 minutes - three hours(may take up to 1 – 2 days)
- Not recommended for long term use as it draws water into the bowel, hydrating and softening the stool thus increasing peristalsis and bowel evacuation.

8.5 Rectal preparations - suppository

- A suppository is a medicated solid formulation prepared for insertion into the rectum to evacuate faeces in the following circumstances:
    - Relief of constipation.
    - Preparation for procedures such as sigmoidoscopy.
- Suppositories should only be used for constipation if all other management strategies have failed and can be useful in the management of chronic constipation in conjunction with oral preparations.
- After insertion, body temperature dissolves the suppository from its solid form to a liquid**.**

8.6 Osmotic laxative

(e.g. Glycerin - Glycerol®)

- Draws water into the faeces via osmosis, hence lubricates the faeces and stimulates peristalsis.
- Onset of action is 5–30 minutes.

8.7 Rectal preparations - enema

- Enemas should only be used for constipation if all other management strategies have not worked.
- An enema is used to administer substances in a liquid form into the rectum for evacuation of faeces in the following situations:
    - Relief of constipation.
    - Faecal incontinence.
    - Preparation for procedures e.g. flexible sigmoidoscopy.
- Due to increased risks of perforation with the administration of an enema it is contraindicated in patients with:
    - Neutropenia or thrombocytopenia due to increased risk of bleeding / infection.
    - Recent colorectal or gynaecological surgery
    - Paralytic ileus or intestinal obstruction - due to decreased / absent peristaltic movements there is a potential risk that absorption of the enema fluid may occur which may contribute to electrolyte imbalance.
    - Recent anal or rectal trauma.
    - Severe colitis.
    - Toxic mega-colon.
    - Undiagnosed abdominal pain.
    - Diarrhoea.
    - Recent radiation therapy to pelvic area.

8.8 Saline Laxative Enema

(e.g. Sodium citrate – Microlax® (first choice of enema)

- Saline enemas contain ions such as magnesium, phosphate, sulphate and citrate which are poorly absorbed.

- As a result of the effect of osmosis, fluid is retained in the colon which results in the stimulation of peristalsis.
- Risk of elevation in serum sodium levels and consequent dehydration.

- Prolonged retention of the enema solution should be avoided, particularly following administration of a phosphate enema.
    - Sodium salts may contribute to or worsen fluid and electrolyte disturbances. Using sodium phosphate may increase the risk of acute renal failure.
    - Risk of electrolyte imbalance and dehydration.
    - Risk of hyponatraemic dehydration.
- Small volume 5mL.
- Sodium phosphate – Fleet®.
- Large volume 133mL.
- Specific consideration should be given before commencing rectal saline laxatives in the following situations:
    - Cardiovascular disease eg: heart failure.
    - Electrolyte disturbance.
    - Medications e.g. diuretics, ACE inhibitors, NSAID's, Angiotensin II antagonists.
    - Elderly.
    - Congenital mega-colon/imperforate anus.

9. Skin care - incontinence associated dermatitis

- Incontinence associated dermatitis (IAD) is defined as inflammation of the skin that occurs when urine or stool comes into contact with sacral, perineal or peri-genital skin.
    - IAD is characterised by epidermal erosion and macerated appearance of the skin.
- There are four factors that contribute to skin breakdown:
    - Excessive moisture / perspiration.
    - High skin pH related to urinary and / or faecal incontinence.
    - Colonisation of skin micro-organisms.
    - Friction.
- Incontinence Associated Dermatitis may present as:
    - Tingling, itching, burning.
    - Pain.
    - Erythema.
    - Crusting and / or scaling.
    - Swelling or ooze.
    - Vesicles.

Sacral Dermatitis

9.1 Assessment

- Assess for signs of moisture, impaired skin integrity, rash/ dryness of the skin.
- Excessive moisture and alkalinity may promote excessive urea production, increasing risk of integument breakdown.

- Skin may appear dry despite over hydration due to excessive cleaning with soap and water.
- Early detection minimises integument breakdown.

9.2 Management

- Cleanse sacral and perineal skin daily and after each incontinence episode using a no rinse cleanser or warm water.
    - Soap is to be avoided due to its high pH which can compromise the natural barrier function of the skin.
- The cleanser selected should be pH neutral and remove contaminants from the integument.
    - Routine cleansing after incontinence promotes skin healing and resists further damage.

9.3 Skin Care

- Moisturising agents and barrier creams are the preferred option to maintain skin integrity as they shield the skin from exposure to irritants and moisture.
- For further advice and product supply contact the continence service or the MO.
- **Ensure to avoid:**
    - Use of talc based powders in skin care management.
    - Use of absorbent products.
    - Consider an under pad.
    - Refer to the Bowel Management Products section.
- Apply appropriate moisturiser and barrier products to the sacral, perineal / perianal area if:
    - The skin is dry or prone to breakdown.
    - Altered skin integrity present.
    - Commence wound management plan as required.

9.4 Documentation

- Assessment, treatment and outcomes of the patient’s bowel management are to be monitored and documented.
- Failure to accurately and legibly record, and understand what is recorded, in the patient health records contribute to a decrease in the quality and safety of patient care.

10. Bowel management products

- The use of products may be indicated after an initial assessment of bowel function has been completed.
- For further continence advice contact the continence service.
- The aim of appropriate product use is to promote:
    - Isolation of faeces from clothing and bedding.
    - Dignity and comfort for the patient.
- Products are **not** to be used as an alternative for toileting programs.

10.1 Assessment

When assessing for product use to manage bowel dysfunction consider:

- Functional ability of the patient.
- Severity of bowel dysfunction.
- Gender.
- Failure with previous treatment.
- Patient's preference and co-morbidity.
- Skin condition in sacral and peri-genital area.
- The long-term use of pads has financial implications for the individual.

Types of Pads

Shaped pad / two-piece system: waterproof backing, secured with adhesive strip. To be worn with firm fitting underwear.
Self-adhesive water proof backing: secured with an adhesive strip. To be worn with firm fitting underwear, can be used for small-moderate faecal loss.
Insert booster pads: to be used in addition with another pad, free from plastic backing, non-adhesive, extends the life of the outer pad, available from the continence service.
Non-adhesive waterproof backing: use with firm fitting underwear or disposable pants. Used for large volume faecal loss, although good manual dexterity is required to manage these pads.
Absorbent body worn / self-contained pants: use for mobile patients unable to manage a two-piece system. These pads are pulled on like underwear.
Side fastening / all-in-one style pads: use for those who are bed / chair bound with intractable incontinence and are unable to be managed by other means. These pads are secured with the aid of adhesive tabs.
Under pad/ disposable absorbent sheet: contraindicated for use with people with severe dermatitis. Use for those who are bed / chair bound with skin excoriation. Check at two hourly intervals to minimise further skin irritation and maintain comfort. **Note:** Tissue bed protector sheets ('Blueys') are **not** a continence aid.

11. Patient monitoring

- Individualised management plan to be documented in the patients' health records as soon as is practicable.
- At a minimum the plan must consider:
    - Patient history and diagnosis for clinical conditions, medications, psychosocial and cultural factors that could influence observations.
    - Presence of comorbidities and treatment.

    - Frequency and specific observations.
    - Site requirements, patient education and consent.

12. Discharge planning

- To assist individuals to maintain good bowel habits on discharge all patients discharged home with a bowel management plan to be educated on healthy bowel habits, lifestyle modifications and the need to commit to these modifications long term.
- A relative or carer is to be involved in education if assisting or caring for the patient at home.
- The discharge plan should be holistic and individualised with the following considerations:
    - Psychological responses to illness and disability.
        - Liaise with the MO as indicated.
    - Patient strengths and weaknesses.
    - Ethnic and cultural beliefs.

On discharge:

- Advise the patient to contact their GP if their bowel function worsens.
- Patient and / or carer given advice on skin care, continence products and information about product choice.
    - The patient may be eligible for funding to assist in the cost of purchasing continence equipment.
- Referral to the continence service as required for clinic appointments, if any concerns or advanced education required, information of any funding schemes available

13. Legislative context

The following documents are required to give effect to this policy:

- Health Department of Western Australia (HDWA) Consent to Treatment Policy OD 0657/16
- HDWA Clinical Handover Policy MP 0095
- East Metropolitan Health Service (EMHS) Patient Identification and Procedure Matching EMHS: 21
- EMHS Discharge Policy EMHS: 99

14. Supporting information

The following documents support the implementation of this policy:

- Bladder Management – Continence Procedure AKG-PRO-0057-17
- Medication Management Procedure AKG-PRO-0255-18

15. Compliance, monitoring and evaluation

Compliance with this policy will be monitored via:

- Monitoring of reported clinical incident data via Datix CIMS.

Department specific findings are to be reported at the relevant departmental meeting for review and identification of any opportunities for improvement.

16. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 2 – Action 2.3 Healthcare rights and informed consent
- The health service organisation has a charter of rights that is:
    - a. Consistent with the Australian Charter of Healthcare Rights[53]
    - b. Easily accessible for patients, carers, families and consumers
- Standard 4 – Action 4.3 Partnering with consumers
- Clinicians use organisational processes from the Partnering with Consumers Standard in medication management to:
    - a. Actively involve patients in their own care
    - b. Meet the patient's information needs
    - c. Share decision-making
- Standard 6 – Action 6.5 Correct identification and procedure matching
- The health service organisation:
    - a. Defines approved identifiers for patients according to best-practice guidelines
    - b. Requires at least three approved identifiers on registration and admission; when care, medication, therapy and other services are provided; and when clinical handover, transfer or discharge documentation is generated
- Standard 6 – Action 6.7 Clinical handover
- The health service organisation, in collaboration with clinicians, defines the:
    - a. Minimum information content to be communicated at clinical handover, based on best-practice guidelines
    - b. Risks relevant to the service context and the particular needs of patients, carers and families
    - c. Clinicians who are involved in the clinical handover

17. References

1. Continence Foundation of Australia

18. Document owner

Director Nursing and Midwifery

Enquiries relating to this guideline may be directed to:

Title: Nursing Coordinator
Department: Hospital Wide

19. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V3	06/06/2024	06/06/2027	Scheduled review

20. Authorisation

This guideline has been authorised and issued by the Director Nursing and Midwifery.

Authorised by	Helen Fullarton – Director Nursing and Midwifery
Authorisation date	31/05/2024
Published date	06/06/2024

Appendix 1: Management of intestinal failure guidelines

- Intestinal failure comprises a group of disorders with many different causes, all of which are characterised by an inability to maintain adequate nutrition via the intestines.
- It results from obstruction, abnormal motility, major surgical resection, congenital defect or disease-associated loss of absorption.
- It is characterised not only by the inability to maintain protein-energy, but also difficulties in maintaining water, electrolyte or micronutrient balance.
- If it persists for more than a few days, it demands treatment with intravenous delivery of nutrients and water (parenteral nutrition).

This guideline is for health professionals caring for patients who:

- Demonstrate an output of 1500mLs via stoma, fistulae and or anus.
- Have symptoms of fluid and electrolyte disturbances which is secondary to high input.
- Require medical treatment, which may include rehydration therapy and / or electrolyte replacement.

Presenting Symptoms

The following signs and symptoms may include but are not limited to:

- >/= 1500mL output via a stoma / fistula / anus.
- Dry mouth with an increased thirst.
- Severe lethargy.
- Headache and / or dizziness.
- Nausea.
- Poor appetite.
- Poor urine output.
- Muscle cramps.

Assessment

- Description of the source and consistency of output.
- Strict fluid balance chart (for inpatients).
- Diet and fluid intake for last 48 hours.
- Length of time of watery output.
- How often has patient been emptying his / her bag, going to the toilet or to use their bowels in a 24 hour period.
- Current medications including dose and frequency.
- History and physical assessment of abdomen.

Management Plan

- The patient should be referred to a senior medical officer and further consultation to be initiated with gastroenterology / surgical teams (refer to specific site protocols).
- The following data must be included in the referral:
    - Diagnosis.
    - Reason for referral.

    - Medical Team details of the person initiating the referral.
- Additional information if available includes:
    - Estimated length of functioning small bowel.
    - Presence / absence of ileocaecal valve.
    - Current nutritional status e.g. TPN, enteral feeding and / or oral diet and fluids.
    - Operative history.
    - Blood tests to determine the patient's hydration status and renal function should be taken including urea and electrolytes (UE), full blood picture (FBP), magnesium (mg+).
- Consultations to help assess and correct nutrient deficiencies and prevent damage to the kidneys, bones and liver.

Dehydration Management

- Intravenous fluids will be required and ordered by the MO.
- If tolerating oral fluids double strength gastrolyte or a solution made up of one teaspoon salt, two teaspoons sugar in 1L of water may be prescribed as this takes the place of all ingested water e.g. tea, coffee and cordial will all need to be made with this fluid mixture if used.
- If a patient with dysphagia is on thickened fluids and requires the fluid mixture:
    - Measure out 1L of their thickened fluids (cordial or water only) level, mildly, moderately and extremely thick.Add one teaspoon of salt and two teaspoons of sugar to the fluid and mix well for 30 seconds.
    - Orally administer the thickened fluid mixture as desired.
    - Note: patients who are on thickened fluids and require the fluid mixture are to have 1L of thickened cordial or water ordered by the Speech Pathologist.
- Oral fluid restriction may also be indicated at the discretion of the MO.

Medications

- Dependent on the patients' output and consistency ongoing assessment and prescription of appropriate medications to be administered as prescribed by the MO e.g. loperamide.

Nutrition

- Patients will need to be reviewed by the Stomal Therapy Nurse and a dietitian.
- Diet modification will be required to train the small intestine to absorb more nutrients.
- Parenteral nutrition may be required.
- If the patient is taking oral diet, they should be encouraged to eat regularly and to add salt to their food.